

Every day you walk into a referral source's office, you carry a number. Your census target. Your admission quota. Your manager's ask for the week. Most reps know the number — very few understand where it comes from or what it actually means for the patients who depend on the branch staying open.

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The People Behind the Census Number

When your leadership sets a census target, they are not picking a number from thin air — they are calculating what the branch needs to pay the people who actually care for patients. Before you can sell effectively, you need to know who those people are and what it costs to keep them in the field.

Medical Director / Attending Physician

The physician who certifies and recertifies patient eligibility every 60–90 days. Without a medical director, the branch cannot bill Medicare and cannot operate. This is typically the highest fixed salary on the org chart, whether there are 10 patients or 100.

Census impact: Fixed cost — present from day one. Low census cannot reduce this role.

RN Case Manager

The clinical quarterback of every patient's care. The RN visits multiple times per week, coordinates the interdisciplinary team, manages symptom crises at 2 a.m., and is the first call when a family is scared. A typical caseload is 12–16 patients per RN. Below that threshold, you are paying for capacity you cannot fill.

Census impact: Scales with census — but in steps. Adding one RN covers 12–16 patients.

Licensed Clinical Social Worker (LCSW)

The social worker navigates financial assistance, family dynamics, advance directives, and the emotional weight of anticipatory grief. Medicare requires social work as part of the interdisciplinary plan of care. This is not optional.

Census impact: One SW serves 25–35 patients depending on acuity and geography.

Chaplain / Spiritual Care Coordinator

Patients and families receive spiritual care regardless of religious affiliation. The chaplain addresses existential questions, facilitates legacy work, and supports the family through and after death. This care makes the hospice experience meaningful in a way no drug or device can replicate.

Census impact: One chaplain typically serves 30–40 patients.

Home Health Aide (HHA)

The HHA visits multiple times per week to assist with bathing, grooming, and personal care. For many patients, these visits are the only human contact they receive outside of family. HHAs also provide critical status updates that alert the clinical team to changes in condition.

Census impact: Caseloads of 6–10 patients per aide, depending on visit frequency.

Volunteer Coordinator

Medicare requires that volunteers provide a minimum of 5% of direct patient care hours. The coordinator recruits, trains, and dispatches volunteers for companionship, respite relief, and errands. Failing this standard creates compliance exposure that jeopardizes the entire Medicare certification.

Census impact: Fixed role — required by regulation regardless of census.

QAPI / Compliance & Administrative Team

Quality Assessment and Performance Improvement is not optional. Chart audits, incident reporting, infection control logs, and survey preparation require dedicated administrative capacity. This team protects the license that lets everyone else do their jobs.

Census impact: Fixed overhead — scales minimally with census.

The Interdisciplinary Team Is Not a Line Item — It Is the Product

When a referral source asks 'What makes your hospice different?', the honest answer is: the depth and consistency of this team showing up for that patient every week. Your census target is what it takes to fund that team. Every patient you admit supports every member of it.

PART II The Daily Cost Reality

What it actually costs to care for one patient

Every Patient Costs Money Every Day

Revenue from Medicare's Routine Home Care (RHC) rate arrives as a flat per-diem — the same amount whether the patient had a crisis visit last night or a routine aide visit. What most reps do not realize is that a significant portion of that per-diem is immediately consumed by direct clinical variable costs before one dollar of payroll is paid.

Cost Category	Typical Range / Day	Base Preset	What Drives It Higher
Pharmacy	\$15 – \$60	\$22.00	Oncology pain management, complex wound care, anti-emetics
DME	\$6 – \$15	\$9.00	Hospital beds, oxygen concentrators, wheelchairs, commodes
Supplies	\$6 – \$20	\$8.00	Wound care supplies, incontinence products, gloves, dressings
Travel	\$4 – \$12	\$7.00	Rural geography, high-traffic metro areas, large territories
Other	\$2 – \$10	\$4.00	Contracted therapy, interpreter services, lab draws
Total Variable Cost	\$33 – \$117	\$50.00	Patient mix and geography are the primary drivers

Base preset values shown. Adjust in the simulator to match your actual market.

Contribution Margin: The Number That Powers Everything

After subtracting variable clinical costs from the RHC per-diem, what remains is the **contribution margin per patient per day**. This is the money available to pay for payroll, overhead, and — if census is high enough — operating profit.

Blended Planning Assumption: \$120 – \$155 / Patient / Day

At national base RHC rates with standard variable costs, most hospice branches operate with a contribution margin between \$120 and \$155 per patient per day depending on patient acuity and length of stay. The Spartan simulator calculates your exact figure based on your actual rates and cost assumptions. This single number drives every threshold in your profitability model.

The Revenue Rate Structure

Medicare pays two RHC rates: a higher rate for days 1–60 of each benefit period, and a lower rate for day 61 and beyond (approximately 21% less). The 2025 national base rates are:

- **Day 1–60:** \$224.62 / day (your CBSA wage index may adjust this)
- **Day 61+:** \$177.27 / day

Patients with longer average lengths of stay spend more time in the lower rate band. The simulator blends these rates based on your average LOS to give you an accurate blended revenue per patient per day for planning purposes.

Why Length of Stay Matters More Than Most Reps Realize

A patient with a 30-day LOS generates 100% of their revenue at the Day 1–60 rate. A patient with a 120-day LOS generates half their revenue at the lower Day 61+ rate. A branch with a higher average LOS has a structurally lower blended revenue per day — which means a higher break-even census. Understanding LOS is understanding your revenue ceiling.

PART III The Numbers

Why your census target is not arbitrary

From Patients to Profitability: The Chain

Branch profitability is not a mystery. It follows a deterministic chain of formulas that connect your average daily census to annual profit or loss. Once you understand this chain, every number your leadership gives you has a clear source — and every conversation you have with a referral source has a clear purpose.

1	Average Daily Census (ADC)	The number of patients actively on service today, on average.
2	Blended Revenue / Day	$ADC \times \text{blended RHC rate, adjusted for LOS mix.}$
3	Annual Revenue	$ADC \times \text{blended rate} \times 365 \text{ days.}$
4	Annual Variable Cost	$ADC \times (\text{pharmacy} + \text{DME} + \text{supplies} + \text{travel} + \text{other}) \times 365.$
5	Annual Payroll	Sum of all FTE salaries required at that census level.
6	Annual Overhead	$\text{Fixed non-payroll costs} \times 12 \text{ months.}$
7	Annual Profit / (Loss)	$\text{Revenue} - \text{Variable Cost} - \text{Payroll} - \text{Overhead.}$
8	Operating Margin %	$\text{Annual Profit} \div \text{Annual Revenue} \times 100.$
9	Break-Even ADC	$\text{Annual Fixed Cost} \div (\text{Contribution Margin} \times 365).$
10	Target Margin ADC	$\text{Fixed Cost} \div (365 \times (\text{Revenue Rate} \times (1 - \text{Target\%}) - \text{Var Cost})).$

Break-Even ADC: The Floor, Not the Goal

Break-even ADC is the census where the branch covers all costs — payroll, variable costs, and overhead — with zero profit remaining. At this level, every clinician is paid, every medication is funded, and the lights stay on. What there is no room for: equipment upgrades, merit raises, marketing investment, or any buffer against a census dip.

At the Base Preset: Break-Even Typically Falls Between ADC 29 – 40

Using national base RHC rates, standard variable costs, and a lean but complete staffing model, most new hospice branches hit break-even somewhere between ADC 29 and ADC 40 — depending on overhead, local wage rates, and market acuity. The simulator computes your exact number. **Break-even is the floor. Your census target is the gap between the floor and your operating margin goal.**

Required Monthly Admissions: The Sales Math

ADC is a stock — the number of patients currently on service. Admissions are the flow that replenishes it. Every month, patients leave the census (through death, discharge, or revocation). The monthly admissions needed to maintain a target ADC is determined entirely by your average length of stay:

Monthly Admissions Formula

Monthly Admissions = (Target ADC × 365) ÷ Average LOS (days) ÷ 12 Example: ADC 60, Average LOS 90 days = $(60 \times 365) \div 90 \div 12 = 21,900 \div 90 \div 12 \approx 20.3$ admissions/month ≈ 4.7 /week

Monthly Admissions Required at Standard LOS (90 days)

Target ADC	Annual Patient-Days	Monthly Admits Needed	Weekly Admits Needed	Marketers Needed (@ 8/mo)
50	18,250	~17 / month	~4 / week	~3 marketers
60	21,900	~20 / month	~5 / week	~3 marketers
80	29,200	~27 / month	~6 / week	~4 marketers
100	36,500	~34 / month	~8 / week	~5 marketers

Assumes average LOS of 90 days. Marketers assumed at 8 admissions/marketer/month. Adjust all variables in the Spartan simulator to match your market.

The ADC 60 Reference Point — 20 Admissions, 5 per Week

A branch targeting ADC 60 with a 90-day average LOS needs approximately **20 admissions per month — roughly 5 per week**. That is not a guess or a quota a manager invented. It is the mathematical replacement rate required to maintain that census given how long your average patient stays on service. Every missed admission is a patient-day that never happens — and a cost the branch bears anyway.

Target Margin ADC: The Real Sales Goal

Most reps are given a census target without understanding that the target is not break-even — it is the census where the branch hits its **operating margin goal**. If your leadership targets a 15% operating margin, the Target Margin ADC is the census where 15 cents of every revenue dollar becomes operating profit.

The formula accounts for the fact that at higher census, your contribution margin must cover not just fixed costs but also the profit margin goal. This always produces a higher threshold than break-even.

The Gap Is Your Sales Job

If break-even is ADC 35 and your target margin ADC is ADC 55, the gap is 20 patients. Those 20 patients are worth — at \$130 contribution margin per patient per day — roughly \$949,000 in annual operating profit. That is what your sales effort creates. That is why your census number is not arbitrary.

Worked Example: Three Admissions per Week

A common new-rep weekly target is three admissions per week. Here is what that actually means for the branch:

- 3 admits/week × 52 weeks = 156 annual admissions
- At 90-day average LOS: steady-state ADC = $156 \times (90 \div 365) \approx$ **38 patients**
- At \$130 contribution/day: annual contribution $\approx$ **\$1.8 million**

- That ADC is near or just above break-even for most branches — not profitable

Three Admissions per Week Keeps the Lights On — Barely

Three admissions per week at 90-day LOS produces approximately ADC 38, which is near break-even for a lean branch and well below any target margin threshold. A coached, consistent rep producing 8–10 admissions per month (roughly 2–2.5 per week) generates roughly the same steady-state ADC when paired with a second rep at the same level — and together they begin creating real operating margin above the break-even floor.

The Spartan Branch Profitability Simulator

The simulator is a live, interactive model of your branch's profitability at any census level. It computes every number in real time using the same Decimal.js precision engine that underlies this guide. There are no approximations — every output is mathematically exact given your inputs.

Input Category	What You Enter	What It Drives
Census & LOS	Target ADC, Average Length of Stay	All revenue, admissions, and sales targets
Revenue Rates	RHC Day 1–60, RHC Day 61+	Blended revenue per day, annual revenue
Variable Clinical Costs	Pharmacy, DME, Supplies, Travel, Other	Contribution margin, variable cost line
Fixed Overhead	Monthly non-payroll overhead	Fixed cost base, break-even ADC
Staffing	Auto-computed from ADC (editable)	Annual payroll, break-even, target margin ADC
Starting Capital	Available launch capital	18-month cash runway simulation
Sales Assumptions	Admissions per marketer per month	Marketer headcount, weekly admission target

Five Ways to Use the Simulator in the Field

1. Frame the Stakes Before a Difficult Conversation

Before meeting with a referral source who has gone quiet, run the simulator at your current census. Show what break-even looks like. Show what the gap costs the branch per month. Reps who can quantify the impact of census stagnation speak with a different level of authority.

2. Translate Admission Targets into Weekly Activity

Use the monthly admissions output to back-calculate your weekly call cycle. If you need 5 admissions per week, and your close rate from qualified referral to admission is 60%, you need roughly 8–9 qualified referral conversations per week. That drives your call schedule — not guesswork.

3. Demonstrate the Value of Your Market to Leadership

Build scenarios for different target ADC levels and show leadership what each milestone unlocks. ADC 40 gets you to break-even. ADC 60 funds a second marketer. ADC 80 creates the margin to invest in quality programs. This is how you get buy-in for growth resources.

4. Anchor Referral Conversations to Patient Impact

The most powerful version of this conversation is not about your numbers — it is about the clinical team that those numbers fund. Share what it costs to keep an RN case manager available 24/7 for every patient. Share what the aide's weekly visit means to a patient who lives alone. Census is not an administrative metric — it is the financial oxygen the clinical team breathes.

5. Model the Cash Runway for New Branch Planning

For reps involved in branch launches or expansion planning, the 18-month runway simulation shows exactly how many months of starting capital the branch has before census must cross break-even. This gives the sales team a hard deadline and a clear ADC milestone to build their ramp toward.

Access the Simulator

The Spartan Branch Profitability Simulator is available to all Spartan Coaching members at spartancoaching.com. Enter your revenue rates, clinical costs, and staffing assumptions to generate a customized profitability model for your branch. Every result can be exported as a PDF for team or leadership distribution.

Glossary of Key Terms

Average Daily Census (ADC)

The average number of patients actively on hospice service on any given day. The single most important driver of branch revenue and profitability.

Average Length of Stay (LOS)

The mean number of days a patient remains on hospice service before death, discharge, or revocation. Drives the blended revenue rate and monthly admission requirement.

Blended Revenue Per Day

The weighted average RHC reimbursement rate accounting for the mix of days at the Day 1–60 rate and the Day 61+ rate based on average LOS. Formula: $((\text{Day1Rate} \times 60) + (\text{Day61Rate} \times (\text{LOS} - 60))) \div \text{LOS}$, for $\text{LOS} > 60$.

Break-Even ADC

The census level at which total revenue exactly covers all variable costs, payroll, and overhead — with zero profit. Formula: $\text{Annual Fixed Cost} \div (\text{Contribution Margin Per Day} \times 365)$.

Contribution Margin Per Day

Revenue per patient per day minus all direct variable clinical costs per day. The amount available to cover fixed costs and generate profit for each patient on service. Typical range: \$120–\$155 per patient per day.

Monthly Admissions Needed

The number of new patient admissions required per month to maintain a target ADC, given the average LOS. Formula: $(\text{Target ADC} \times 365) \div \text{Average LOS} \div 12$.

Operating Margin %

Annual operating profit as a percentage of annual revenue. Formula: $(\text{Revenue} - \text{Variable Cost} - \text{Payroll} - \text{Overhead}) \div \text{Revenue} \times 100$. Healthy range for a hospice branch: 12–18%.

Routine Home Care (RHC)

Medicare's standard per-diem reimbursement rate for hospice patients receiving care at home. Paid as a flat daily rate regardless of the volume of services provided on that day.

Target Margin ADC

The census level at which the branch achieves its goal operating margin percentage. Always higher than break-even ADC. This — not break-even — is the actual sales target. Formula: $\text{Annual Fixed Cost} \div (365 \times (\text{Revenue Rate} \times (1 - \text{Target Margin\%}) - \text{Variable Cost}))$.

Variable Clinical Cost

Direct costs that scale with the number of patients on service: pharmacy, DME, clinical supplies, travel, and other per-patient expenses. Expressed as dollars per patient per day.

Weekly Admissions Needed

Monthly admissions needed $\times 12 \div 52$. The weekly admission rate required to maintain target ADC. This is the operational target that drives call cycles, territory plans, and marketer headcount.

18-Month Cash Runway

A simulation of how long a branch's starting capital can absorb monthly losses during the census ramp period, assuming a linear ADC build over the first 12 months followed by steady-state operation at target ADC for months 13–18.

A Final Word

The reps who win long-term are not the ones who show up most often — they are the ones who show up with the clearest understanding of what is at stake. Every admission you generate funds a nurse, a social worker, a chaplain, an aide. Every referral source you develop protects the branch's ability to serve the next patient who needs it. Your census number is not arbitrary. It never was.

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